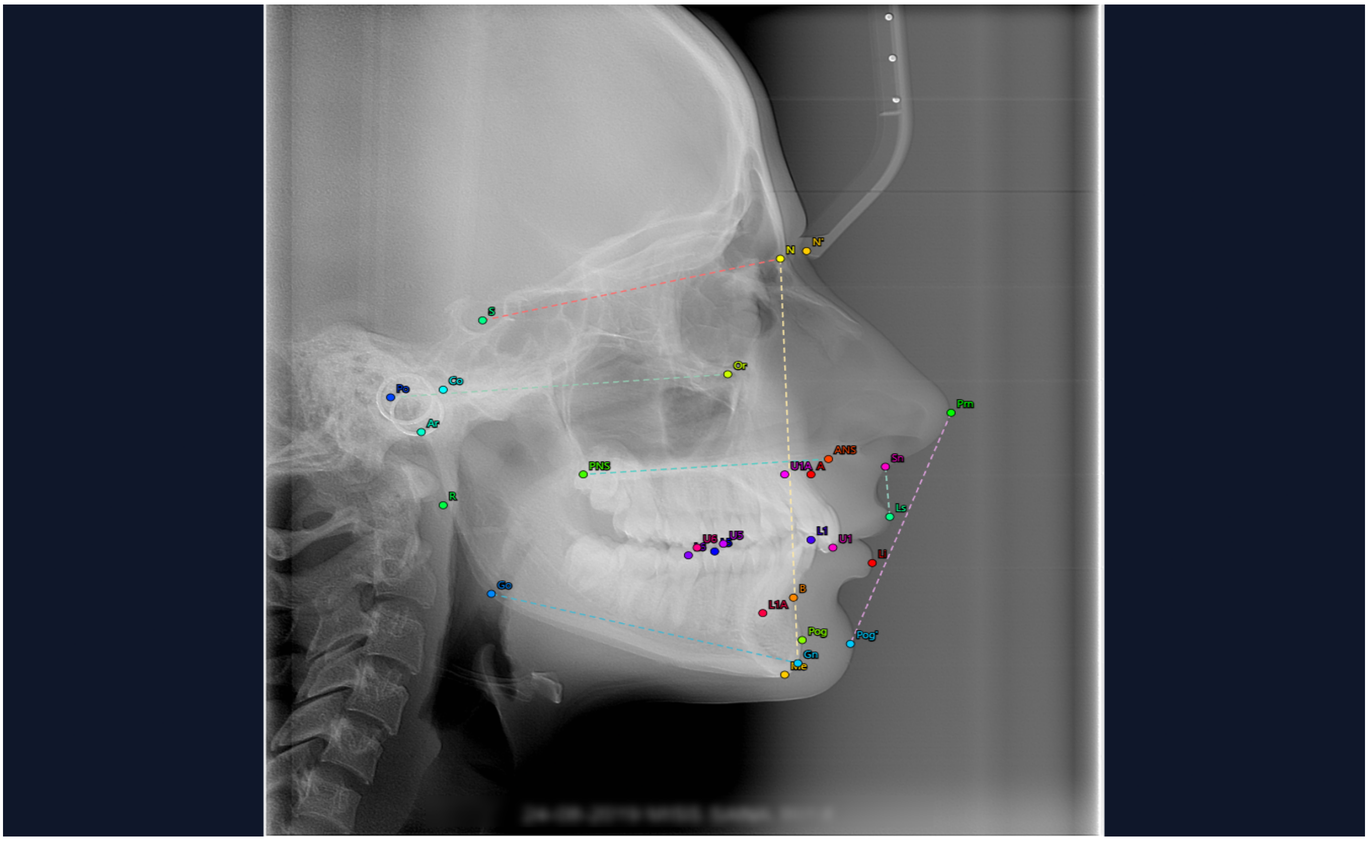
Cephalometric Analysis Report

April 2, 2026

AI-powered landmark detection · 29 landmarks

cks2ip8fz2ao00yuf0p085exq.png



Steiner Analysis

Measurement	Value	Normal	Status
SNA	83.8°	82° ± 2°	Normal
SNB	78.7°	80° ± 2°	Normal
ANB	5.2°	2° ± 2°	Above
Mandibular Plane	27.7°	32° ± 3°	Below

Ricketts Analysis

Measurement	Value	Normal	Status
Interincisal Angle	120°	130° ± 6°	Below
Lower Facial Height %	52.7%	55% ± 5%	Normal
Mandibular Plane Angle	27.7°	26° ± 4°	Normal
Facial Depth Angle	91.6°	90° ± 3°	Normal

McNamara Analysis

Measurement	Value	Normal	Status
Maxillary Length	89.2mm	91 ± 6 mm	Normal
Mandibular Length	110.8mm	120 ± 7 mm	Below
Anterior Facial Height	111.3mm	120 ± 5 mm	Below
Facial Convexity	4.3mm	2 ± 2 mm	Above

AI Clinical Diagnosis

Skeletal Pattern

Skeletal Class II : ANB of 5.2° (Steiner norm 2°) indicates a significant maxillomandibular discrepancy with the maxilla positioned ahead of the mandible.

SNA of 83.8° (Steiner norm 82°) is within normal limits — maxilla is normally positioned relative to the cranial base.

SNB of 78.7° (Steiner norm 80°) is within normal limits — mandible is normally positioned relative to the cranial base.

Both SNA and SNB are within normal limits — the elevated ANB may reflect a **dentoalveolar** rather than skeletal discrepancy.

Mixed growth pattern indicators : Steiner GoGn-SN 27.7° is below the mean of 32°, suggesting a horizontal growth tendency; Ricketts FH-MP 27.7° is within normal range (norm 26° ± 4°). Note: Steiner uses the SN plane as reference while Ricketts uses the Frankfort Horizontal (FH). These different reference planes can yield differing assessments for the same patient.

Convex profile , supported by Ricketts Convexity of Point A at 4.3mm (high positive value suggests a Class II skeletal pattern; norm 2mm at age 9, decreases with age).

Key Findings

McNamara Proportional Analysis : Maxillary length 89.2mm, mandibular length 110.8mm — **proportional** per Table 10-1 (expected 111-113mm for a 89.2mm midface). Difference of 21.6mm is within the expected range (22-24mm).

Anterior Facial Height : 111.3mm is decreased (norm 120 ± 5mm).

Interincisal Angle : 120° is below the Steiner norm of 131° (Table 7-1). The angle is **acute** , indicating **proclined incisors** (tipped forward/labially). Per Steiner, teeth with acute interincisal angles often require **uprighting** .

Clinical Summary

The cephalometric analysis reveals a significant maxillary protrusion (ANB 5.2°), indicative of a dentoalveolar rather than skeletal discrepancy. The mixed growth pattern suggests both horizontal and vertical tendencies, with the maxilla positioned ahead of the mandible in terms of SNA-SN angle (Steiner norm 82°) and FH-MP angle (Ricketts norm 26°). The convex profile further supports a Class II skeletal relationship.

The patient's interincisal angle is acute at 120°, indicating proclined incisors that require uprighting. Proportional analysis shows a slight discrepancy in maxillary and mandibular lengths (89.2mm vs 110.8mm), with the anterior facial height being reduced to 111.3mm from the norm of 120±5mm, suggesting a proclined incisor relationship necessitating uprighting.

Given these findings, potential treatment considerations include orthodontic uprighting of the incisors and possibly interceptive or comprehensive orthodontic therapy to address the dentoalveolar discrepancy, while monitoring for any signs of skeletal discrepancies. Close follow-up will be essential to assess growth patterns and adjust treatment plans accordingly.

[WARNING] Clinical Disclaimer: This AI-generated analysis is intended as a diagnostic aid only. Cephalometric measurements from different analyses (Steiner, Ricketts, McNamara) may yield differing interpretations due to different reference planes and norms. All findings should be correlated with clinical examination, patient history, and soft tissue assessment before arriving at a definitive diagnosis or treatment plan. This report does not constitute a professional orthodontic diagnosis.